



POLICY

Patients' Freedom of Lawful Communication

Scope (Staff):	All NMHS Mental Health (MH) Staff
Scope (Area):	All NMHS MH Inpatient Areas

1. Aim

A patient may only have their right to freedom of lawful communication limited or restricted by a Consultant Psychiatrist under sections 261 & 262 of the [Mental Health Act \(WA\) 2014](#) (MHA 2014) This includes the completion of, and management of, processes pertaining to the "Restriction of Freedom of Communication" Form 12C.

2. Risk

Adherence to this policy ensures the provision of appropriate consideration for medical staff when restricting the freedom of lawful communication for inpatients within an authorised hospital. There is a risk if this policy is not followed then this may contravene the MHA 2014.

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Principles

The decision to limit or restrict a patient's right to freedom of lawful communication may only be made by the Consultant Psychiatrist in consultation with the treating team and, where possible, the patient, carers and nominated others, and must only be made if the Consultant Psychiatrist is satisfied that making the order is in the best interests of the patient.

Lawful communications include:

- Seeing and speaking with other people in the hospital to the extent that this is reasonable,
- Having uncensored communications with people including:
 - receiving visits;
 - sending and receiving telephone calls, and
 - sending and receiving mail and electronic communications.
- Receiving visits from a third party, at all reasonable times.

These lawful communications may only be restricted by the treating Consultant Psychiatrist under sections 261 and 262 of the MHA 2014.

4. Restricting Communication with the Legal Representative or Advocate

The Consultant Psychiatrist can limit or restrict visits from the legal representative or advocate (refer s. 262 (3) of the MHA 2014) only if the Consultant Psychiatrist is satisfied that:

- there is serious risk to the safety of the legal representative or advocate, and;
- There are no other steps that could reasonably be taken to reduce the risk to the legal representative or advocate.

The Consultant Psychiatrist cannot limit or restrict other forms of communication with the legal representative or advocate (refer s. 262 (4) of the MHA 2014) i.e. telephone calls.

5. Procedure

A Form 12C, "Restriction on freedom of communication" will be completed (refer s. 262 (5) & (6) of the MHA 2014) by the Consultant Psychiatrist via PSOLIS and a copy filed in the medical record, and provided to the patient and to their carer, close family member and/or other personal support person/nominated persons by the Consultant Psychiatrist or delegate.

An order is current for 24 hours. After this time, the form will lapse. The order must be reviewed every 24 hours and the decision made by the Consultant Psychiatrist for the order to:

- Remain in force (for a further 24 hours),
- Be amended, or;
- Be revoked.

The decision will be documented on a Form 12C.i (SHMHMR987) and filed in the medical record and a copy provided to the patient and to their carer, close family member and/or other personal support person/nominated persons, by the Consultant Psychiatrist or delegate.

For sites using **Digital Medical Record (DMR)**, store the original 12C Form in the patients "Work in Progress" file for later scanning into the DMR. Once scanned, the 12C Form will appear in the Legal Folder of the patient's DMR.

The Consultant Psychiatrist will document in the patient's Progress Notes (PMR7) explaining why the patient's freedom of lawful communication has been limited or restricted.

The Shift Coordinator will indicate in the ward report book or equivalent, that a patient's right to freedom of lawful communication has been limited or restricted to ensure timely review.

Clinical staff will be responsible for removing personal mobile devices and restricting access to the ward public/other available means of communication for patients on restrictions. Any specific management strategies required should be reflected in the patients treatment, support and discharge plan (TSDP).

6. Compliance and Evaluation

All Form 12C's are to be reviewed every 24 hours by a consultant psychiatrist.

All initial form 12C's are logged through PSOLIS and accessed by the mental health advocacy service and reported to services quarterly.

Related internal policies, procedures and guidelines
DoH Clinical Handover Policy DoH Clinical Handover Matrix DoH WA Clinical Handover Guideline

References
Mental Health Act 2014

Useful resources (including related forms)
Clinicians' Practice Guide to the Mental Health Act 2014 Form 12C - Restriction on Freedom of Communication Form Form 12C attachment

Policy Sponsor	Adult Inpatient Medical Co-Director MHPHDS				
Policy Contact	A/Program Manager Adult Inpatient Services MHPHDS				
Date First Issued:	11/07/2012	Last Reviewed:	13/08/2024	Review Date:	13/08/2027
Approved by:	Adult Inpatient Leadership Committee			Date:	July 2024
Endorsed by:	Adult Inpatient Medical Co-Director MHPHDS			Date:	13/08/2024
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety			☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration	
Chief Psychiatrist's Standards for Clinical Care:	Consumer and Carer Involvement in Individual Care,				
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Document Version Control			
Date	Version	Author	Notes
08/04/2019	2.0	Acting Nurse Coordinator, Graylands Hospital	Schedule Review: updated template, references and hyperlinks, updated policy to reflect correct procedure and monitoring of lawful communication and any restrictions to this under the Mental Health Act 2014.
07/09/2022	3.0	Acting Program Manager, Adult Inpatient Services	Template update, no changes to content, hyperlinks updated
13/08/2024	4.0	Adult Inpatient Co-Director	Scheduled review, template and hyperlinks updated, no content change

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISI9P0119)

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